Inpatient Hospital Discharge Summary

Hospital Identification & Admission

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Facility: Metro Heart & Vascular Institute, Super Specialty Hospital, Gurugram, NCR

Admission Date: 14-Oct-2024 | Discharge Date: 20-Oct-2024 (Hospitalization Duration: 6 Days)

Attending Cardiologist: Dr. Ashish Sachdeva, MD, DM (Cardiology), FACC

Department: Cardiac Care Unit (CCU) & Interventional Cardiology

Patient Name: Mr. Devendra Varma | Age: 54 years | Gender: Male

1. Final Diagnoses

1. Acute Anterior Wall ST-Elevation Myocardial Infarction (STEMI).

2. Status Post-Emergency Primary Percutaneous Coronary Intervention (PCI) with Drug-Eluting Stent (DES) to Proximal Left Anterior Descending (LAD) Artery (14-Oct-2024).

3. Type 2 Diabetes Mellitus (Duration: 8 years, poorly controlled).

4. Diabetic Nephropathy (Stage 3a Chronic Kidney Disease).

5. Essential Systemic Hypertension (Stage 2).

6. Moderate Left Ventricular Systolic Dysfunction (LVEF: 42%).

2. Clinical Presentation & Emergency Course

History: 54-year-old male with long-standing poorly controlled diabetes and hypertension presented with sudden onset acute crushing retrosternal chest pain radiating to left jaw and back, associated with cold diaphoresis, nausea, and shortness of breath lasting 3.5 hours.

Emergency Room Vitals: BP 164/98 mmHg, Pulse 102 bpm irregular, SpO2 93% on ambient air.

Emergency 12-Lead ECG: Marked 3.5 mm ST-segment elevation in leads V1 to V5 with hyperacute T waves and reciprocal ST depressions in inferior leads (II, III, aVF) diagnostic of Acute Anterior STEMI.

Emergency Cardiac Biomarkers: High Sensitivity Troponin-I: 18.40 ng/mL (Reference normal: < 0.04 ng/mL; severely elevated). Creatine Kinase-MB: 84 U/L (Reference: < 25 U/L).

3. Coronary Angiography and Intervention

Coronary Angiography (14-Oct-2024, 02:45 AM): Right femoral artery access.

- LMCA: Unremarkable.

- LAD: 95% thrombotic, critical eccentric stenosis in proximal LAD with TIMI-1 distal flow.

- LCx: 45% non-flow limiting calcified plaque in mid circumflex.

- RCA: 50% diffuse disease in mid segment.

Intervention Procedure: Primary PCI to proximal LAD. Lesion crossed with 0.014 Runthrough wire, predilated with 2.5 x 12 mm balloon. Deployed Xience Sierra 3.5 x 28 mm Everolimus-eluting stent at 14 atm. Post-dilated with 3.75 x 12 mm NC balloon at 18 atm. Excellent angiographic result with 0% residual stenosis and restoration of TIMI-3 distal coronary flow.

4. In-Hospital 2D Echocardiography

Study Date: 19-Oct-2024 (Pre-discharge)

Findings: Left Ventricular Ejection Fraction (LVEF) = 42% by Simpson's biplane method.

Wall Motion: Akinesia of apex and anterior apical wall; hypokinesia of mid anterior and anteroseptal segments.

Mitral Valve: Mild functional mitral regurgitation. Left ventricular diastolic dysfunction Grade 1.

5. Discharge Medications & Secondary Prevention Plan

1. Tab Aspirin 75 mg once daily (post-lunch).
2. Tab Ticagrelor 90 mg twice daily (DAPT for minimum 12 months).
3. Tab Metoprolol Succinate ER 50 mg once daily.
4. Tab Rosuvastatin 40 mg once daily at bedtime (high-intensity statin).
5. Tab Ramipril 2.5 mg once daily (titrated to renal function).
6. Tab Metformin 1000 mg twice daily + Tab Glimepiride 2 mg twice daily.

Discharge Advice: Cardiac rehabilitation, strict avoidance of physical exertion, strict low salt and diabetic diet, urgent nephrology review for proteinuria.

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